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April 21, 2025

COMMITTEE SUBSTITUTE
FOR ENGROSSED
HOUSE BILL NO. 2051

By: Stinson and Archer of the
House

and

Thompson of the Senate

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[ supervised physician practice - supervision
requirements - rules - licensure - penalties -
collaborative practice arrangement - disciplinary
actions - identification - certification course -
exemptions - codification - effective date ]
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BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 479.1 of Title 59, unless there is created a duplication in numbering, reads as follows:

Sections 1 through 11 of this act shall be known and may be cited as the "Supervised Physicians Act".

SECTION 2. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 479.2 of Title 59, unless there is created a duplication in numbering, reads as follows:

As used in the Supervised Physicians Act:

1 1. "Medical school graduate" means any person who has graduated
2 from a legally chartered medical school recognized by the Oklahoma
3 State Regents for Higher Education, or from a school of osteopathic
4 medicine as defined in Section 631 of Title 59 of the Oklahoma
5 Statutes;

6 2. "Supervised physician" means a medical school graduate who:

7 a. is a resident and citizen of the United States,

8 b. has successfully completed Step 1 and Step 2 of the
9 United States Medical Licensing Examination (USMLE),
10 or the equivalent of Step 1 and Step 2 of any other
11 medical licensing examination or combination of
12 examinations that is approved by the State Board of
13 Medical Licensure and Supervision or the State Board
14 of Osteopathic Examiners, within the same calendar
15 year as the person's graduation from a medical school
16 or school of osteopathic medicine,

17 c. has successfully graduated from an Oklahoma medical
18 school with a doctorate of medicine or a doctorate of
19 osteopathic medicine; notwithstanding any other
20 provisions of the Supervised Physicians Act, this
21 subparagraph is the controlling provision for the
22 location of the medical school the supervised
23 physician may be a graduate of, and
24

d. has entered into a supervised physician collaborative practice arrangement as defined in paragraph 3 of this section;

3. "Supervised physician collaborative practice arrangement" means an agreement between an Oklahoma licensed supervising physician and a supervised physician in good standing that meets the requirements of the Supervised Physicians Act; and

4. "Supervising physician" means the physician tasked with overseeing or delegating the activities of the medical services rendered by a supervised physician through a practice agreement between a supervising physician performing procedures or directly or indirectly involved with the treatment of a patient, and the supervised physician working jointly toward a common goal of providing services. Delegation shall be defined by the practice arrangement. The physical presence of the delegating physician is not required as long as the supervising physician and supervised physician are or can be easily in contact with each other by telecommunication. At all times a supervised physician shall be considered an agent of the supervising physician. The supervising physician shall meet the following criteria:

a. have possession of a full and unrestricted Oklahoma license to practice medicine, with United States Drug Enforcement Administration (DEA) and Oklahoma State

1 Bureau of Narcotics and Dangerous Drugs Control

2 (OBNDD) permits, and

3 b. be trained and fully qualified in the field of the
4 supervised physician's specialty.

5 SECTION 3. NEW LAW A new section of law to be codified
6 in the Oklahoma Statutes as Section 479.3 of Title 59, unless there
7 is created a duplication in numbering, reads as follows:

8 Supervised physicians shall be subject to the supervision
9 requirements established in any controlling federal law, any
10 supervision requirements provided in the Supervised Physicians Act,
11 and any supervision requirements established by the State Board of
12 Medical Licensure and Supervision or the State Board of Osteopathic
13 Examiners. Supervised physicians are not subject to any additional
14 supervision requirements, other than the supervision requirements
15 provided in this section.

16 SECTION 4. NEW LAW A new section of law to be codified
17 in the Oklahoma Statutes as Section 479.4 of Title 59, unless there
18 is created a duplication in numbering, reads as follows:

19 A. The State Board of Medical Licensure and Supervision and the
20 State Board of Osteopathic Examiners shall independently promulgate
21 rules, with the input and assistance of designated institutional
22 officers and graduate medical education departments at the state's
23 medical schools:
24

1 1. To establish the process for temporary licensure of
2 supervised physicians, supervision requirements, and additional
3 requirements for supervised physician collaborative practice
4 arrangements;

5 2. To set fees in an amount greater than or equal to the total
6 costs necessary to facilitate the supervised physician collaborative
7 practice arrangement each year; and

8 3. To address any other matters necessary to protect the public
9 and discipline the profession.

10 B. A supervised physician's temporary license issued pursuant
11 to the Supervised Physicians Act and the rules promulgated by the
12 State Board of Medical Licensure and Supervision or the State Board
13 of Osteopathic Examiners shall only be valid for two (2) years from
14 the date of the supervised physician's medical school graduation and
15 is not subject to renewal. The State Board of Medical Licensure and
16 Supervision or the State Board of Osteopathic Examiners may deny an
17 application for temporary licensure or suspend or revoke the
18 temporary license of a supervised physician for violation of the
19 standards provided in the Oklahoma Allopathic Medical and Surgical
20 Licensure and Supervision Act or the Oklahoma Osteopathic Medicine
21 Act, or such other standards of conduct established by the State
22 Board of Medical Licensure and Supervision or the State Board of
23 Osteopathic Examiners by rule.
24

1 C. The Supervised Physicians Act shall not be construed to be
2 an alternative pathway to full licensure. The license created in
3 the Supervised Physicians Act shall only be temporary for the amount
4 of time allowed in the Supervised Physicians Act.

5 SECTION 5. NEW LAW A new section of law to be codified
6 in the Oklahoma Statutes as Section 479.5 of Title 59, unless there
7 is created a duplication in numbering, reads as follows:

8 A supervised physician shall clearly identify himself or herself
9 as a supervised physician and shall clearly be identified as a
10 supervised physician on his or her name tag or lab coat. A
11 supervised physician shall not practice, or attempt to practice,
12 without a supervised physician collaborative practice arrangement,
13 except as otherwise provided in the Supervised Physicians Act.

14 SECTION 6. NEW LAW A new section of law to be codified
15 in the Oklahoma Statutes as Section 479.6 of Title 59, unless there
16 is created a duplication in numbering, reads as follows:

17 The licensed supervising physician collaborating with a
18 supervised physician shall be responsible for directly supervising
19 the activities of the supervised physician and shall accept full
20 responsibility for the services provided by the supervised
21 physician.

22 SECTION 7. NEW LAW A new section of law to be codified
23 in the Oklahoma Statutes as Section 479.7 of Title 59, unless there
24 is created a duplication in numbering, reads as follows:

1 A. The Supervised Physicians Act applies to all supervised
2 physician collaborative practice arrangements. To be eligible to
3 practice as a supervised physician, a temporary licensed supervised
4 physician shall enter into a supervised physician collaborative
5 practice arrangement with a supervising physician by the end of the
6 calendar year of his or her graduation from medical school and no
7 later than thirty (30) days after the date on which the supervised
8 physician obtains initial licensure.

9 B. Only a physician licensed by the State Board of Medical
10 Licensure and Supervision or the State Board of Osteopathic
11 Examiners may enter into a supervised physician collaborative
12 practice arrangement with a supervised physician. Supervised
13 physician collaborative practice arrangements shall take the form of
14 a written agreement that includes mutually agreed-upon protocols and
15 any standing orders for the delivery of services. Supervised
16 physician collaborative practice arrangements may delegate to a
17 supervised physician the authority to prescribe, administer, or
18 dispense drugs and provide treatment, as long as the delivery of the
19 services is within the scope of the supervising physician's practice
20 and is consistent with the supervised physician's skill, training,
21 and competence and the skill, training, and competence of the
22 supervising physician; except that a supervised physician shall not
23 prescribe controlled dangerous substances. The supervising
24

1 physician shall be board-certified in the specialty that the
2 supervised physician is practicing.

3 C. The supervised physician collaborative practice arrangement
4 shall contain the following provisions:

5 1. Complete names, home and business addresses, and telephone
6 numbers of the supervising physician and the supervised physician;

7 2. A requirement that the supervised physician practice at the
8 same location as the supervising physician;

9 3. A requirement to prominently display a disclosure statement
10 at the practice location informing patients that they may be seen by
11 a supervised physician and advising patients that they have the
12 right to see the supervising physician;

13 4. All specialty or board certifications of the supervising
14 physician and all certifications of the supervised physician;

15 5. The manner of collaboration between the supervising
16 physician and the supervised physician, including how the
17 supervising physician and the supervised physician will engage in
18 collaborative practice consistent with each professional's skill,
19 training, education, and competence;

20 6. A requirement that the supervised physician shall not
21 provide patient care during an absence of the supervising physician
22 for any reason;

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1 7. A list of all other supervised physician collaborative
2 practice arrangements of the supervising physician and the
3 supervised physician;

4 8. The duration of the supervised physician collaborative
5 practice arrangement between the supervising physician and the
6 supervised physician; and

7 9. A provision describing the time and manner of the
8 supervising physician's review of the supervised physician's
9 delivery of services. The provision shall require that the
10 supervising physician shall review one hundred percent (100%) of the
11 charts documenting the supervised physician's delivery of services.

12 D. A copy of the supervised physician collaborative practice
13 arrangement shall be kept at the business address of the supervising
14 physician and the supervised physician. Copies of the arrangement
15 shall be provided upon request.

16 SECTION 8. NEW LAW A new section of law to be codified
17 in the Oklahoma Statutes as Section 479.8 of Title 59, unless there
18 is created a duplication in numbering, reads as follows:

19 A. The State Board of Medical Licensure and Supervision and the
20 State Board of Osteopathic Examiners shall independently promulgate
21 rules regulating the use of supervised physician collaborative
22 practice arrangements for supervised physicians. The rules shall:

23 1. Specify the methods of treatment that may be covered by the
24 supervised physician collaborative practice arrangement; and

1 2. Require review of the services provided under a supervised
2 physician collaborative practice arrangement.

3 B. A supervising physician may enter into a supervised
4 physician collaborative practice arrangement with not more than
5 three supervised physicians at the same time, subject to any other
6 laws or rules of this state that address limits on supervision.

7 SECTION 9. NEW LAW A new section of law to be codified
8 in the Oklahoma Statutes as Section 479.9 of Title 59, unless there
9 is created a duplication in numbering, reads as follows:

10 A. Within thirty (30) days of any change to a supervised
11 physician collaborative practice arrangement, including the initial
12 practice arrangement, a supervised physician and a supervising
13 physician shall report to the State Board of Medical Licensure and
14 Supervision or the State Board of Osteopathic Examiners whether the
15 physician is engaged in a supervised physician collaborative
16 practice arrangement, and shall report to the physician's licensing
17 board the name of each supervised physician with whom the physician
18 has entered into an arrangement. Each board may make the
19 information available to the public. The State Board of Medical
20 Licensure and Supervision and the State Board of Osteopathic
21 Examiners shall track the reported information and may routinely
22 conduct reviews or inspections to ensure that the arrangements are
23 being carried out in compliance with the Supervised Physicians Act.
24 Copies of the supervised practice arrangements shall be provided

1 upon request of the State Board of Medical Licensure and Supervision
2 or the State Board of Osteopathic Examiners. Copies shall be stored
3 at the business address of the supervising physician and the
4 supervised physician.

5 B. A contract or other agreement shall not require a physician
6 to act as a supervising physician for a supervised physician against
7 the physician's will. A physician has the right to refuse to act as
8 a supervising physician, without penalty, for a particular
9 supervised physician. A contract or other agreement shall not limit
10 the supervising physician's authority over any protocols or standing
11 orders, or delegate the physician's authority to a supervised
12 physician. However, this subsection does not authorize a physician
13 in implementing protocols, standing orders, or delegation to violate
14 applicable standards for safe medical practice established by a
15 hospital's medical staff.

16 C. A contract or other agreement shall not require a supervised
17 physician to serve as a supervised physician for any supervising
18 physician against the supervised physician's will. A supervised
19 physician has the right to refuse to collaborate, without penalty,
20 with a particular physician.

21 D. All supervising physicians and supervised physicians under a
22 supervised physician collaborative practice arrangement shall wear
23 identification badges while acting within the scope of the
24 arrangement. The identification badges shall prominently display

1 the licensure status of the supervising physician and the supervised
2 physician.

3 SECTION 10. NEW LAW A new section of law to be codified
4 in the Oklahoma Statutes as Section 479.10 of Title 59, unless there
5 is created a duplication in numbering, reads as follows:

6 A. The supervising physician shall complete a certification
7 course, which may include material on the laws pertaining to the
8 professional relationship. The certification course shall be
9 approved by the State Board of Medical Licensure and Supervision or
10 the State Board of Osteopathic Examiners.

11 B. A supervised physician collaborative practice arrangement
12 shall supersede current hospital licensing regulations governing
13 hospital medication orders under protocols or standing orders for
14 the purpose of delivering inpatient or emergency care within a
15 hospital as defined in Section 1-701 of Title 63 of the Oklahoma
16 Statutes, if the protocols or standing orders have been approved by
17 the hospital's medical staff and pharmaceutical therapeutics
18 committee.

19 SECTION 11. NEW LAW A new section of law to be codified
20 in the Oklahoma Statutes as Section 479.11 of Title 59, unless there
21 is created a duplication in numbering, reads as follows:

22 The provisions of the Supervised Physicians Act only apply to
23 inpatient hospital settings in facilities that are accredited by the
24 Accreditation Council for Graduate Medical Education.

1 SECTION 12. AMENDATORY 59 O.S. 2021, Section 492, is
2 amended to read as follows:

3 Section 492. A. Every person shall be regarded as practicing
4 allopathic medicine within the meaning and provisions of ~~this act~~
5 the Oklahoma Allopathic Medical and Surgical Licensure and
6 Supervision Act, who shall append to his or her name the letters
7 "M.D.", "Physician", or any other title, letters, or designation
8 which represent that such person is a physician, or who shall for a
9 fee or any form of compensation diagnose and/or treat disease,
10 injury, or deformity of persons in this state by any allopathic
11 legend drugs, surgery, manual, or mechanical treatment unless
12 otherwise authorized by law.

13 B. A hospital, as defined in Section 1-701 of Title 63 of the
14 Oklahoma Statutes, or a related institution as such terms are
15 ~~defined in Section 1-701 of Title 63 of the Oklahoma Statutes~~, which
16 has the principal purpose or function of providing hospital or
17 medical care, including, but not limited to, any corporation,
18 association, trust, or other organization organized and operated for
19 such purpose, may employ one or more persons who are duly licensed
20 to practice medicine in this state without being regarded as itself
21 practicing medicine within the meaning and provisions of this
22 section. The employment by the hospital or related institution of
23 any person who is duly licensed to practice medicine in this state
24 shall not, in and of itself, be considered as an act of

unprofessional conduct by the person so employed. Nothing provided herein shall eliminate, limit, or restrict the liability for any act or failure to act of any hospital, any hospital's employees, or persons duly licensed to practice medicine.

C. The definition of the practice of medicine and surgery shall include, but is not limited to:

1. Advertising, holding out to the public, or representing in any manner that one is authorized to practice medicine and surgery in this state;

2. Any offer or attempt to prescribe, order, give, or administer any drug or medicine and surgery for the use of any other person, except as otherwise authorized by law;

3. a. ~~any~~ Any offer or attempt, except as otherwise authorized by law, to prevent, diagnose, correct, or treat in any manner or by any means, methods, devices, or instrumentalities except for manual manipulation any disease, illness, pain, wound, fracture, infirmity, defect, or abnormal physical or mental condition of any person, including the management of pregnancy and parturition, except as otherwise authorized by law,

b. ~~except~~ Except as provided in subsection D of this section, performance by a person within or outside of this state, through an ongoing regular arrangement, of

1 diagnostic or treatment services, including, but not
2 limited to, stroke prevention and treatment, through
3 electronic communications for any patient whose
4 condition is being diagnosed or treated within this
5 state by a physician duly licensed and practicing in
6 this state. A person who performs any of the
7 functions covered by this subparagraph submits himself
8 or herself to the jurisdiction of the courts of this
9 state for the purposes of any cause of action
10 resulting from the functions performed, and

11 c. ~~nothing~~ Nothing in the Oklahoma Allopathic Medical and
12 Surgical Licensure and Supervision Act shall be
13 construed to affect or give jurisdiction to the State
14 Board of Medical Licensure and Supervision over any
15 person other than medical doctors or persons holding
16 themselves out as medical doctors;

17 4. Any offer or attempt to perform any surgical operation upon
18 any person, except as otherwise authorized by law; and

19 5. The use of the title Doctor of Medicine, Physician, Surgeon,
20 Physician and Surgeon, Dr., M.D., or any combination thereof in the
21 conduct of any occupation or profession pertaining to the
22 prevention, diagnosis, or treatment of human disease or condition
23 unless, where appropriate, such a designation additionally contains
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1 the description of another branch of the healing arts for which one
2 holds a valid license in this state.

3 D. The practice of medicine and surgery, as defined in this
4 section, shall not include:

5 1. A student while engaged in training in a medical school
6 approved by the Board or while engaged in graduate medical training
7 under the supervision of the medical staff of a hospital or other
8 health care facility approved by the state medical board for such
9 training, except that a student engaged in graduate medical training
10 shall hold a license issued by the Board for such training;

11 2. Any person who provides medical treatment in cases of
12 emergency where no fee or other consideration is contemplated,
13 charged, or received;

14 3. A commissioned medical officer of the ~~armed forces~~ Armed
15 Forces of the United States or medical officer of the United States
16 Public Health Service or the United States Department of Veterans
17 Affairs ~~of the United States~~ in the discharge of official duties
18 and/or within federally controlled facilities; and provided that
19 such person shall be fully licensed to practice medicine and surgery
20 in one or more jurisdictions of the United States; provided further
21 that such person who holds a medical license in this state shall be
22 subject to the provisions of the Oklahoma Allopathic Medical and
23 Surgical Licensure and Supervision Act;

1 4. Any person licensed under any other act when properly
2 practicing in the healing art for which that person is duly
3 licensed;

4 5. The practice of those who endeavor to prevent or cure
5 disease or suffering by spiritual means or prayer;

6 6. Any person administering a domestic or family remedy to a
7 member of such person's own family;

8 7. Any person licensed to practice medicine and surgery in
9 another state or territory of the United States who renders
10 emergency medical treatment or briefly provides critical medical
11 service at the specific lawful direction of a medical institution or
12 federal agency that assumes full responsibility for that treatment
13 or service and is approved by the Board;

14 8. Any person who is licensed to practice medicine and surgery
15 in another state or territory of the United States whose sole
16 purpose and activity is limited to brief actual consultation with a
17 specific physician who is licensed to practice medicine and surgery
18 by the Board, other than a person with a special or restricted
19 license; or

20 9. The practice of any other person as licensed by appropriate
21 agencies of this state, provided that such duties are consistent
22 with the accepted standards of the person's profession and the
23 person does not represent himself or herself as a Doctor of
24

1 Medicine, Physician, Surgeon, Physician and Surgeon, Dr., M.D., or
2 any combination thereof.

3 E. Nothing in the Oklahoma Allopathic Medical and Surgical
4 Licensure and Supervision Act shall prohibit:

5 1. The service rendered by a physician's unlicensed trained
6 assistant, if such service is rendered under the supervision and
7 control of a licensed physician pursuant to Board rules, provided
8 such rules are not in conflict with the provisions of any other
9 healing arts licensure act or rules promulgated pursuant to such
10 act; or

11 2. The service of any other person duly licensed or certified
12 by the state to practice the healing arts.

13 F. Nothing in the Oklahoma Allopathic Medical and Surgical
14 Licensure and Supervision Act shall prohibit services rendered by
15 any person not licensed by the Board and practicing any
16 nonallopathic healing practice.

17 G. Nothing in the Oklahoma Allopathic Medical and Surgical
18 Licensure and Supervision Act shall be construed as to require a
19 physician to secure a Maintenance of Certification (MOC) as a
20 condition of licensure, reimbursement, employment, or admitting
21 privileges at a hospital in this state. For the purposes of this
22 subsection, "Maintenance of Certification (MOC)" shall mean a
23 continuing education program measuring core competencies in the
24

1 practice of medicine and surgery and approved by a ~~nationally-~~
2 ~~recognized~~ nationally recognized accrediting organization.

3 H. 1. A physician licensed in good standing to practice in
4 another state shall be exempt from the licensure requirements of the
5 Oklahoma Allopathic Medical and Surgical Licensure and Supervision
6 Act if either of the following apply:

7 a. the physician has a written or oral agreement with a
8 sports team to provide care to team members and
9 coaching staff traveling with the team for a specific
10 sporting event to take place in this state, or

11 b. the physician has been invited by a national sport
12 governing body to provide services to team members and
13 coaching staff at a national sport training center in
14 this state or to provide services at an event or
15 competition in this state which is sanctioned by the
16 national sport governing body so long as:

17 (1) the physician's practice is limited to that
18 required by the national sport governing body,
19 and

20 (2) the services provided by the physician are within
21 the physician's scope of practice.

22 2. Nothing in this subsection shall be construed as to permit a
23 physician exempt by the provisions of this subsection to:
24

1 a. provide care or consultation to any person residing in
2 this state, other than a person specified in paragraph
3 1 of this subsection, or

4 b. practice at a medical facility licensed by the State
5 Department of Health.

6 3. An exemption obtained under subparagraph a of paragraph 1 of
7 this subsection shall be valid while the physician is traveling with
8 the sports team, provided no exemption shall be longer than ten (10)
9 days in duration for each respective sporting event. A maximum of
10 twenty (20) additional days per sporting event may be granted upon
11 prior request to the State Board of Medical Licensure and
12 Supervision, provided no physician shall be exempted more than
13 thirty (30) additional days in a calendar year.

14 4. An exemption obtained under subparagraph b of paragraph 1 of
15 this subsection shall be valid during the time certified by the
16 national sport governing body, provided no exemption shall be for
17 longer than thirty (30) days.

18 5. The State Board of Medical Licensure and Supervision may
19 enter into agreements with medical licensing boards of other states
20 to implement the provisions of this subsection. Agreements may
21 include procedures for reporting potential medical license
22 violations.

23 6. The State Board of Medical Licensure and Supervision shall
24 promulgate rules to implement the provisions of this subsection.

SECTION 13. AMENDATORY 59 O.S. 2021, Section 622, is amended to read as follows:

Section 622. A. 1. Except as otherwise provided by this section, it shall be unlawful for any person to practice as an osteopathic physician and surgeon in this state, without a license to do so, issued by the State Board of Osteopathic Examiners; provided, that any license or certificate issued under the laws of this state, authorizing its holder to practice osteopathic medicine, shall remain in full force and effect. Persons who hold themselves out as osteopathic physicians in this state without a license issued by the State Board of Osteopathic Examiners shall submit themselves to the jurisdiction of the State Board of Osteopathic Examiners.

2. Osteopathic physicians engaged in postgraduate training beyond the internship year, also known as PGY-1, shall be licensed. Osteopathic physicians engaged in the internship or PGY-1 year may be eligible for a resident training license.

3. Osteopathic physicians engaged in interventional pain management pursuant to the Oklahoma Interventional Pain Management and Treatment Act shall be licensed by the State Board of Osteopathic Examiners.

B. 1. A person within or outside of this state who performs through electronic communications diagnostic or treatment services within the scope of practice of an osteopathic physician and surgeon, including, but not limited to, stroke prevention and

1 treatment, for any patient whose condition is being diagnosed or
2 treated within this state shall be licensed in this state, pursuant
3 to the provisions of the Oklahoma Osteopathic Medicine Act.

4 However, in such cases, a nonresident osteopathic physician who,
5 while located outside this state, consults on an irregular basis
6 with a physician who is located in this state is not required to be
7 licensed in this state.

8 2. Any osteopathic physician licensed in this state who engages
9 in the prescription of drugs, devices, or treatments via electronic
10 means may do so only in the context of an appropriate
11 ~~physician/patient~~ physician-patient relationship wherein a proper
12 patient record is maintained including, at the minimum, a current
13 history and physical.

14 3. Any commissioned medical officer of the ~~armed forces~~ Armed
15 Forces of the United States or medical officer of the United States
16 Public Health Service or the ~~Veterans Administration of the~~ United
17 States Department of Veterans Affairs, in the discharge of official
18 duties and/or within federally controlled facilities, who is fully
19 licensed to practice osteopathic medicine and surgery in one or more
20 jurisdictions of the United States shall not be required to be
21 licensed in this state pursuant to the Oklahoma Osteopathic Medicine
22 Act, unless the person already holds an osteopathic medical license
23 in this state pursuant to the Oklahoma Osteopathic Medicine Act. In
24

1 such case, the medical officer shall be subject to the Oklahoma
2 Osteopathic Medicine Act.

3 4. A person who performs any of the functions covered by this
4 subsection submits themselves to the jurisdiction of the courts of
5 this state for the purposes of any cause of action resulting from
6 the functions performed.

7 C. A hospital, as defined in Section 1-701 of Title 63 of the
8 Oklahoma Statutes, ~~or a related institution, as such terms are~~
9 ~~defined in Section 1-701 of Title 63 of the Oklahoma Statutes~~, which
10 has the principal purpose or function of providing hospital or
11 medical care, including, but not limited to, any corporation,
12 association, trust, or other organization organized and operated for
13 such purpose, may employ one or more persons who are duly licensed
14 to practice osteopathic medicine in this state without being
15 regarded as itself practicing osteopathic medicine within the
16 meaning and provisions of this section. The employment by the
17 hospital or related institution of any person who is duly licensed
18 shall not, in and of itself, be considered as an act of
19 unprofessional conduct by the person so employed. Nothing provided
20 herein shall eliminate, limit, or restrict the liability for any act
21 or failure to act of any hospital, any hospital's employees, or
22 persons duly licensed to practice osteopathic medicine.

23 D. Nothing in the Oklahoma Osteopathic Medicine Act shall be
24 construed as to require an osteopathic physician to secure an

Osteopathic Continuous Certification (OCC) as a condition of licensure, reimbursement, employment, or admitting privileges at a hospital in this state. For the purposes of this subsection, "Osteopathic Continuous Certification (OCC)" shall mean a continuing education program measuring core competencies in the practice of medicine and surgery and approved by a ~~nationally recognized~~ nationally recognized accrediting organization.

E. 1. An osteopathic physician licensed in good standing to practice in another state shall be exempt from the licensure requirements of the Oklahoma Osteopathic Medicine Act if either of the following apply:

a. the physician has a written or oral agreement with a sports team to provide care to team members and coaching staff traveling with the team for a specific sporting event to take place in this state, or

b. the physician has been invited by a national sport governing body to provide services to team members and coaching staff at a national sport training center in this state or to provide services at an event or competition in this state which is sanctioned by the national sport governing body so long as:

(1) the physician's practice is limited to that required by the national sport governing body, and

1 (2) the services provided by the physician are within
2 the physician's scope of practice.

3 2. Nothing in this subsection shall be construed as to permit
4 an osteopathic physician exempt by the provisions of this subsection
5 to:

6 a. provide care or consultation to any person residing in
7 this state, other than a person specified in paragraph
8 1 of this subsection, or

9 b. practice at a medical facility licensed by the State
10 Department of Health.

11 3. An exemption obtained under subparagraph a of paragraph 1 of
12 this subsection shall be valid while the physician is traveling with
13 the sports team, provided no exemption shall be longer than ten (10)
14 days in duration for each respective sporting event. A maximum of
15 twenty (20) additional days per sporting event may be granted upon
16 prior request to the State Board of Osteopathic Examiners, provided
17 no physician shall be exempted more than thirty (30) additional days
18 in a calendar year.

19 4. An exemption obtained under subparagraph b of paragraph 1 of
20 this subsection shall be valid during the time certified by the
21 national sport governing body, provided no exemption shall be for
22 longer than thirty (30) days.

23 5. The State Board of Osteopathic Examiners may enter into
24 agreements with medical and osteopathic licensing boards of other

1 states to implement the provisions of this subsection. Agreements
2 may include procedures for reporting potential medical license
3 violations.

4 6. The State Board of Osteopathic Examiners shall promulgate
5 rules to implement the provisions of this subsection.

6 SECTION 14. This act shall become effective January 1, 2026.

7 COMMITTEE REPORT BY: COMMITTEE ON HEALTH AND HUMAN SERVICES
8 April 21, 2025 - DO PASS AS AMENDED BY CS
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